Foundations NRSG 231 · Midterm Study Guide (v2)

Midterm: Wed Oct 21, 2026, 8:30 AM, Room B137. Examplify, 90 min, 50 points. Question types: multiple choice, short answer, select all that apply, hot spot (click on a photo) and drag and drop. Arrive 15 min early and bring your phone and name badge.

Built only from her materials: Ch. 25 Asepsis slides, the Ch. 34 Activity lecture, Ch. 26 Vital Signs slides, Ch. 8 Communication slides, the Communication Scenario role-play deck, Weekly Skills FALL 26, and the Midterm FAQ "Key Content Areas" list. There is **no textbook content**. Organized by the FAQ topics, in FAQ order.

NOT IN SLIDES marks an FAQ item her slides don't answer. Get it from class notes, the lab checklist or the vital sign cheat sheet; this guide doesn't fill it in. The FAQ itself says **"MEMORIZE the vital sign cheat sheet."**

1 · Ch. 25 Asepsis & Infection Control

Chain of infection and how to break it [Slides 3, 4, 14, 16]

Infection = a disease state from pathogens in or on the body. All **six links** must be present [Slide 3]:

Link	Her examples	Break the chain [Slide 4 figure]
1. Infectious agent	Bacteria (E. coli, staph, strep); viruses (HIV, cold, flu, COVID-19, hepatitis A/B/C); fungi (athlete's foot, ringworm, yeast); parasites (malaria via mosquito) [Slide 5]	Hand hygiene, sterilization, antibiotics/antimicrobials
2. Reservoir	People, animals, soil, food, water, milk, objects [Slide 6]	Transmission-based precautions; sterilization or disposable supplies
3. Portal of exit	Respiratory, GI, GU tracts, skin breakdown, blood & drainage [Slide 11]	Dry intact dressing, hand hygiene, gloves if contact with body fluids, cover nose and mouth when sneezing
4. Means (mode) of transmission	**Contact**: direct (touching, kissing, sex) or indirect via a **vector** (living creature) or **fomite** (inanimate object) [Slide 8]. **Droplet**: organisms >5 µm, heavy, land on surfaces. **Airborne**: organisms <5 µm, stay in the air [Slide 9]	Hand hygiene, pesticides to eliminate vectors, adequate refrigeration
5. Portal of entry	Respiratory, GI, GU tracts, skin breakdown, blood & tissues [Slide 7]	Hand hygiene, gloves, masks and protective gear, proper sharps disposal
6. Susceptible host	Multiple wounds/poor skin integrity (= multiple portals of entry), pH of GI/GU/skin, WBC count, age/sex/heredity, immunity (natural vs acquired vs passive), fatigue, nutrition, pre-existing illness,	Immunizations; screen health care staff

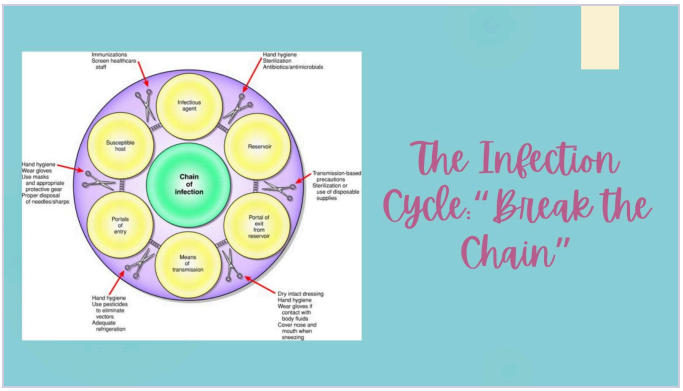
	certain meds, stress, invasive or indwelling devices [Slide 12]	

Slide 14 (APIC poster), 5 steps: 1 Stop at the source · 2 Block the portal of exit · 3 Interrupt mode of transmission · 4 Protect portal of entry · 5 Increase host's defenses.

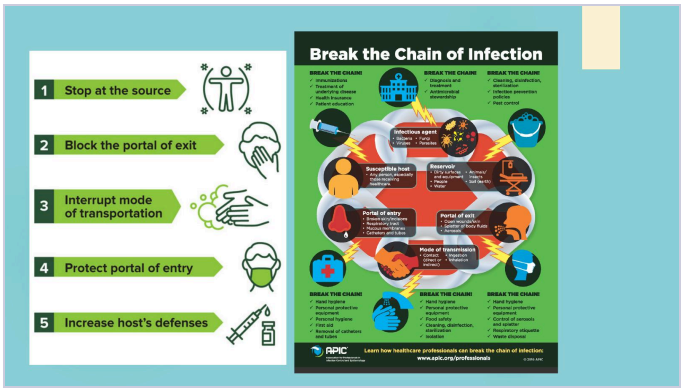
Her practice question [Slide 16]: breaking the chain at the *mode of transmission* = **wearing gloves during wound care**. Antibiotics act on the agent, and nutrition strengthens the host.

Case study, Jimin [Slide 13]: leg laceration, IV in his arm, unvaccinated for COVID, dehydration, stress and fatigue all make him a susceptible host.

What makes an organism thrive [Slide 15]: quantity, virulence, the patient's immune system, how long the contact lasted, and what type of contact it was.



Slide 4: break the chain



Slide 14: APIC poster

Stages of infection [Slides 17–18]

1. **Incubation**: e.g. common cold 1–2 days, tetanus 2–21 days
2. **Prodromal**: hours to days; vague symptoms
3. **Full stage of illness**
4. **Convalescent**: illness resolves, recovery, return to baseline. There may be a temporary or permanent change from previous health.

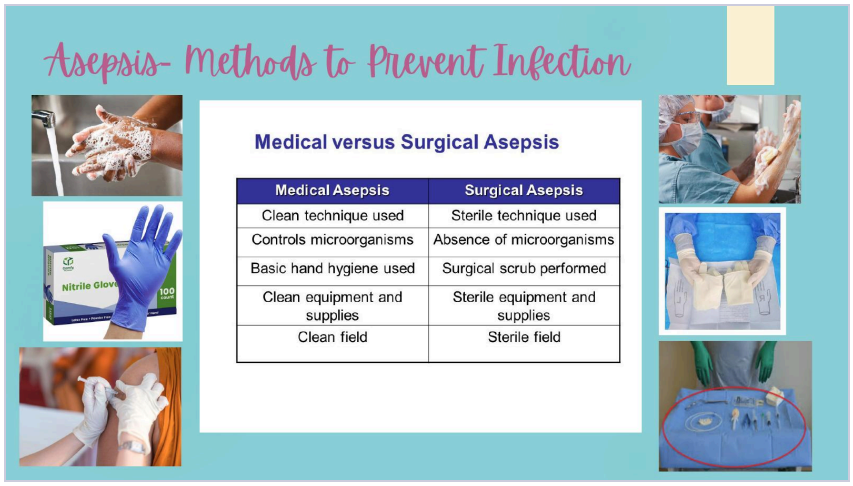
Can someone cycle through the same infection again? Yes, e.g. **HSV** [Slide 17]. **Her knowledge check [Slide 18]:** a student who is "too tired to keep my head up" with a low-grade fever is in the **prodromal** stage.

Medical asepsis vs surgical asepsis, and asepsis practices [Slides 19–21]

Medical asepsis: "clean technique"	Surgical asepsis: "sterile technique"
Reduces the number of pathogens; controls microorganisms	**Eliminates** pathogens; absence of microorganisms
Basic hand hygiene	Surgical scrub
Clean equipment and supplies; clean field	Sterile equipment and supplies; sterile field
Practices: hand hygiene, gloves, never put soiled linen on the floor, avoid raising dust, clean-to-dirty, put dirty linen directly in the hamper, pour contaminated liquids into the drain or "hopper" to avoid splashing	Creates and keeps a pathogen-free environment. Required for Foley catheters and central line placement/dressing changes. Sterile gloves, mask, hair net, gown, etc.

WHO Five Moments for Hand Hygiene [Slide 21]: 1 Before touching a patient · 2 Before a clean or aseptic procedure · 3 After body fluid exposure risk · 4 After touching a patient · 5 After touching patient surroundings.

NOT IN SLIDES Slide 24 only *asks* how long to wash with soap and water, how to use sanitizer, which is more effective and when sanitizer isn't appropriate. The answers aren't on the slides.



Slide 20: medical vs surgical asepsis

MDROs: what they are and how to prevent them [Slides 23, 29]

- **VRE** (vancomycin-resistant Enterococci): feces, urine, blood
- **MRSA** (methicillin-resistant Staph aureus): nares, skin, GI tract
- **CRE** (carbapenem-resistant Enterobacteriaceae)
- **C. diff** (Clostridium difficile): feces

Prevention on her slides: a patient infected or colonized with an MDRO goes on **contact precautions**, with a private room, gown and gloves, glove changes after touching infective material, and equipment cleaned and not shared [Slide 29].

Isolation precautions [Slides 26–31]

Type	Her slide details
Standard [26]	All hospitalized patients regardless of diagnosis. Applies to blood, all body fluids, secretions and excretions **except sweat**, non-intact skin and mucous membranes. Newer additions: respiratory hygiene/cough etiquette, safe injection practices, and a mask for high-risk prolonged spinal canal procedures.
Transmission-based [27]	Used **in addition to** standard precautions for suspected airborne, droplet or contact pathogens. Don PPE when entering, doff when leaving. A disease can have more than one route.
Droplet [28]	Large-particle droplets · private room · **door can stay open** · surgical mask and gloves
Contact [29]	MDRO · private room · **gown and gloves** · change gloves after infective material · clean equipment, don't share
Airborne [30]	Spread through the air · private room · **negative air pressure** · **door closed**, patient stays in the room · **respirator mask** · surgical mask on the patient for transport
Reverse/neutropenic [31]	Severely immunocompromised/immunosuppressed; any cancer treatment or transplant; **WBC below 500 cells/mm³**

PPE [Slides 25, 27; Weekly Skills Week 2]

PPE = **gloves, gown, masks, protective eyewear** [Slide 25]. Don PPE entering a contact or droplet room and doff when leaving [Slide 27]. Week 2 lab and ATI checklists: Performing Hand Hygiene, **Applying PPE, Removing PPE**; TracPrac: standard precautions with hand hygiene, **Don and Doff PPE** [Skills p. 1].

NOT IN SLIDES The donning and doffing **order** isn't on her slides or the skills sheet. Use the ATI "Applying PPE / Removing PPE" checklist from Week 2 lab.

HAIs: what they are and how to prevent them [Slide 22]

Health care–associated infections, also called hospital-acquired or nosocomial infections: **CAUTI** (catheter-associated UTI), **SSI** (surgical site infection), **CLABSI** (central line–associated bloodstream infection), **VAP** (ventilator-associated pneumonia).

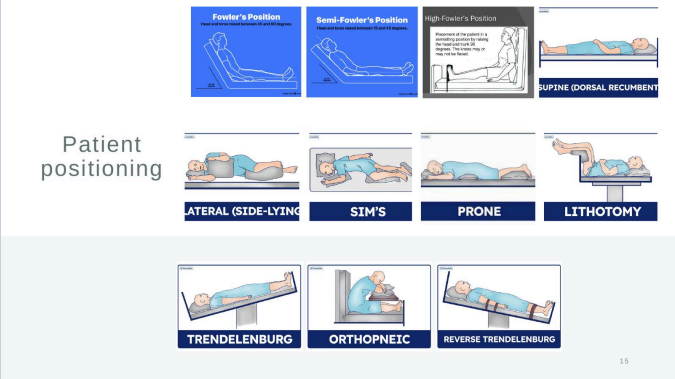
Tie-ins from her slides: indwelling devices make a patient more susceptible [Slide 12]. Foley catheters and central lines require **surgical asepsis** [Slide 19]. Slide 24 asks what you can conclude when a patient with a device shows infection signs. **NOT IN SLIDES** She doesn't give an HAI-specific prevention list beyond those points. Her answer to "best way to prevent infection?" (Slide 32) is shown only as a question, but the slides keep coming back to hand hygiene.

2 · Ch. 34 Safe Patient Handling (Activity lecture + Week 3 lab)

Positioning & body mechanics [Slides 3–7, 12]

- **Alignment/posture**: best musculoskeletal balance with no undue strain on joints, muscles, tendons or ligaments. **Balance**: center of gravity close to a wide base of support. Spreading the feet and flexing the hips and knees increases balance and reduces strain [3].
- **Coordinated movement** uses the major, stronger muscle groups. **Postural reflexes** (inner ear, vision, autonomic contractions) maintain position and equilibrium [4].
- Factors affecting movement: developmental level; physical health (hip dysplasia, torticollis, scoliosis; dwarfism, osteoporosis, muscular dystrophy; arthritis, trauma; stroke, spinal cord injury); mental health (depression); lifestyle (diet, sedentary, smoking); culture. ADLs = eating, bathing, dressing, toileting [5].
- **Ergonomics**: designing equipment and tasks to fit the worker so injuries are prevented [6].
- **Preventing injury [7]**: erect posture; use the **longest, strongest muscles of the arms and legs** (the back is weaker); engage the core (glutes down, abs up); **work close to the object**; **face the direction of movement, don't twist**; **push rather than pull**, arms close; feet shoulder-width apart with knees flexed. Video: **do not bend at the waist** [8].

Positions [Slide 12]: Fowler's (head of bed 45–60°), semi-Fowler's (15–45°), high-Fowler's (about 90°, knees may or may not be flexed), supine (dorsal recumbent), lateral (side-lying), Sim's, prone, lithotomy, Trendelenburg, orthopneic, reverse Trendelenburg.



Slide 12: patient positioning



Slide 11: mobility devices

Ensuring safe patient handling [Slides 9–11]

1. Assess the patient: ability to understand and cooperate, diagnosis and capabilities, braces/devices; encourage them to help as much as they can
2. Enough staff present
3. Clear clutter and check access and devices
4. Use a screening/assessment tool
5. Choose the equipment
6. Plan and communicate with the team and the patient
7. Give a prescribed analgesic before the move if the patient is in pain
8. Elevate the bed as needed
9. **Lock the wheels**

10. Keep the patient safe: avoid friction, support good alignment (don't grab), move in smooth rhythmic motions

Causes of handling injuries [10]: uncoordinated lifts, high exertion in awkward positions, manual lifting without devices, repetitive lifting and repositioning, lifting while fatigued, standing for long periods, bed-to-gurney/wheelchair transfers, repositioning in bed, and patients who are cognitively impaired, dependent, obese or unable/unwilling to help. **Devices [11]:** friction-reducing sheets, standing assist, gait belts, lateral assist device, powered full-body lifts.

FAQ lab skills [Slide 13; Weekly Skills Week 3]

Week 3 lab: ambulation/assisted fall, wheelchairs, positioning/Hoyer lift, lateral transfers. ATI checklists: Moving and Turning Clients in Bed, Safe and Effective Transfer Techniques, Transferring Clients. TracPrac: ambulate a client; transfer to chair, commode or wheelchair; reposition in bed with safe body mechanics [Skills p. 1–2].

NOT IN SLIDES Step-by-step procedures for **ambulation with safe (assisted) fall, wheelchair transfer, lateral transfer to gurney and repositioning in bed** aren't on the slides. Slide 13 only lists them. Use the ATI checklists and lab notes.

3 · Vital Signs Pt. 1 & 2

The vital signs are temperature, pulse, respirations, BP, pulse oximetry and pain level [Slide 3]. **When to assess [4]:** on admission; per policy; any change in condition; loss of consciousness; before and after surgery or invasive procedures; before and after risky activity (e.g. first ambulation after surgery); before meds that affect heart or lungs; whenever the patient reports a change, pain or distress. A baseline set lets you spot changes, and a client's own normal can differ from standard values [26].

Temperature pathophysiology [Slides 5–10]

- **Hypothalamus** controls thermoregulation by conserving heat or increasing heat loss. Temperature = heat produced minus heat lost [5].
- Affected by time of day, biological sex, age, activity, health and environment. **Core** sites: intracranial, intrathoracic, intra-abdominal. Cells work best at **36–37.5 °C (98.6 °F)** [5].
- **Circadian**: lower in the morning, **peaks 1500–1800**. Extreme cold leads to hypothermia and extreme heat to hyperthermia [6]. Slide 6 asks whether older adults run higher or lower and about women; **NOT IN SLIDES** the answers aren't given.
- **Fever (pyrexia)**: febrile vs afebrile. **>40 °C (>104 °F) dangerous**; **≥41 °C (106 °F) = hyperpyrexia, a medical emergency**. Signs: loss of appetite, headache, hot dry skin, thirst, muscle aches, fatigue [7]. Slide 8 asks which other vital sign rises with fever; **NOT IN SLIDES** the answer isn't given.
- **Treating fever [9]**: fever is an immune defense, so treatment aims for comfort and preventing complications. Drugs: antibiotics for bacterial infection, antipyretics. Non-drug: cool bath, cooling blankets, removing extra clothing and blankets, oral fluids.
- **Hypothermia [10]**: poor coordination, slurred speech, amnesia, hallucinations, stupor; RR, HR and BP **decrease**. Treat by raising body temperature: warm blankets, warm fluids, Bair Hugger.

All methods to measure temperature [Slides 11–12; Skills p. 2]

Route	Key points	Time
Oral	Posterior sublingual pocket (near the carotid artery). No hot/cold drinks or smoking for 20 min before. Patient must be awake and alert. Not for small children (they may bite down).	3 min
Axillary	Bulb in the center of the axilla, lower arm across the chest. Non-invasive and good for children but less accurate. **0.5 °C lower than oral.**	5–10 min
Rectal	Side-lying with the upper leg flexed; lubricated, inserted 1–1½ in (adult). Use when oral is unsafe or inaccurate (unconscious, disoriented, irrational). **0.5 °C higher than oral.**	2–3 min
Tympanic (ear)	Close to the hypothalamus, so sensitive to core changes. **Adult: pull the pinna up and back.** Fast and easy. Cerumen impaction or otitis media distorts the reading.	2–3 sec

Thermometer types: infrared, tympanic, electronic oral/axillary, electronic rectal attachment [11]. The Week 4 skills sheet also lists a **temporal** video and says rectal is "discussion" only. **NOT IN SLIDES** Temporal technique isn't on her slides.



Slide 11: thermometers

First-quarter scope for abnormal temperature

From the FAQ: most interventions are out of your scope, but you can always **recheck and notify the provider (or primary nurse)**. Her non-drug measures (cool bath, cooling blanket, removing extra blankets, oral fluids; warm blankets and fluids for hypothermia) are on Slides 9–10. **NOT IN SLIDES** She doesn't say which of these a first-quarter student may do on their own. Giving an antipyretic is medication administration.

Full 3-part radial pulse assessment [Slides 13–15]

- The pulse is the wave of blood when the **left ventricle contracts**. Sites: carotid, brachial, radial, femoral, popliteal, posterior tibial, dorsalis pedis [13].
- Use your **2nd, 3rd or 4th fingers, never the thumb**. **Count 30 sec × 2**. If irregular, count radial for **1 full minute**, then apical for a full minute. **Apical** = PMI, auscultated for a full minute; peripheral pulses are palpated [14].
- **The 3 parts [15]: (1) Rate**, 60–100, average 80 bpm · **(2) Rhythm**, regular or irregular · **(3) Amplitude**: weak/thready, full/bounding, or imperceptible.
- **NOT IN SLIDES** Slide 15 leaves the tachycardia and bradycardia definitions blank to fill in during class.
- FAQ on abnormal pulse: most interventions are out of scope; **recheck the pulse and notify the provider or primary nurse**.

Electronic vital signs machine

Week 4 lab: the electronic VS machine for **SpO_2, temperature and HR**, plus manual pulse and respirations. TracPrac: pulse oximetry (intermittent or continuous), electronic BP, electronic pulse, temperature (axillary, oral, rectal-discussion), manual pulse, manual respirations, and the documentation exercise [Skills p. 2]. **NOT IN SLIDES** How to operate the machine isn't on her slides.

Pulse oximetry [Slide 23]: non-invasive estimate of arterial O_2 saturation (SpO_2). Probes go on the finger, ear, nose or toe. Remove thick nail polish. Wait until the reading is steady and the pulse display is at full strength. With continuous monitoring, check the skin under the probe and rotate it. **NOT IN SLIDES** She gives no normal SpO_2 range on the slides.

Full respiratory rate assessment [Slides 16–17]

- Gas exchange of O_2 and CO_2, under both autonomic and voluntary control. Inspiration = inhalation; expiration = exhalation.

- Count **right after the pulse while still holding the wrist**, so the patient doesn't know you're counting [16].
- **Rate**: one inhale plus one exhale = 1 cycle. **Normal 12–20/min (adult)**. Tachypnea = abnormal increase, bradypnea = abnormal decrease, apnea = no breathing.
- **Depth**: normal (deep, even), shallow (minimal rise), SOB (shallow and rapid). **Rhythm**: regularity, with little variation in pauses. **Character**: **dyspnea** = difficult or labored breathing [17].
- **NOT IN SLIDES** Slide 16's blank "difficulty breathing lying down" term is never filled in on the slides.

Abnormal RR: positioning, O_2 and scope

From the FAQ: for **SOB or dyspnea, put the patient in high-Fowler's or orthopneic position**. Both positions are pictured on Ch. 34 Slide 12; high-Fowler's = upright at about 90°. **Oxygen administration is NOT within your scope.** Reassess and notify the provider. In her Slide 25 case, the question after a high RR is which other assessment shows oxygenation, i.e. pulse oximetry [Slide 23]. **NOT IN SLIDES** The SOB-to-high-Fowler's rule and the O_2 scope limit come from the FAQ only, not the Vital Signs slides.

Blood pressure [Slides 19–21]

BP is the force of blood against vessel walls. **Systolic** = highest pressure during contraction; **diastolic** = lowest pressure during relaxation. Factors: position changes (**orthostatic hypotension is common in the elderly**), drugs, food, exercise, age, weight, biological sex, race, stress [19].

Her BP values, exactly as on Slide 20	
Normal	Less than 120/80
Hypotension	Lower than 90 mmHg systolic or 60 mmHg diastolic
Prehypertension (elevated)	Systolic between 120–129 and diastolic of 80
Hypertension Stage 1	130–139 (systolic), 80–89 (diastolic)
Hypertension Stage 2	>140 (systolic), >90 (diastolic)
Hypertensive crisis	>180

> Note: these are her Slide 20 values word for word, and her exam will use them. Her wording overlaps at exactly 80 diastolic, which she lists under both "elevated" and Stage 1, and at 140/90, where Stage 2 says ">140, >90". The vital sign cheat sheet she says to memorize may settle these edges.

Technique [21]: cuff size by **arm circumference, not age**; cuff covers **40%** of the arm circumference and the bladder covers **2/3** of the arm; arm supported **at heart level, palm up** (above the heart gives a **false low**). **Cuff too wide = false low; too narrow = false high; too loose = false high.** Korotkoff sounds = sounds of blood flowing through an artery as the cuff is deflated after occluding it. **Limb alerts:** no BP on a limb with a PICC line, an AV fistula, or on the mastectomy side.

Manual 2-step BP checklist [Weekly Skills Weeks 5–6]

Week 5 lab: **pt 1 = brachial pulse + PSP (palpated systolic pressure)**; **pt 2 = auscultation**. Week 6 = the 2-step manual BP check-off (see the remediation protocol if not passed). Practice on

as many adult "patients" as you can. Manual BP goes into TracPrac only after you pass the check-off [Skills p. 3]. **NOT IN SLIDES** The full LLUSN Manual BP checklist steps aren't in these sources; use the checklist and the ATI 2-step video.

Vital sign ranges & terminology (from her slides only)

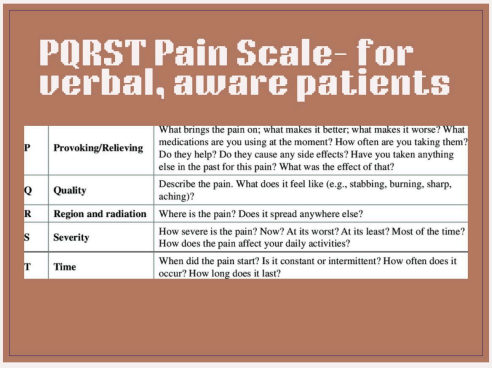
VS	Normal (adult)	Terms
Temp	36–37.5 °C (98.6 °F) [5]	Febrile/afebrile, pyrexia, hyperpyrexia ≥41 °C (106 °F), hypothermia, hyperthermia
Pulse	60–100, avg 80 [15]	Tachycardia/bradycardia (left blank on the slide), weak/thready, bounding, imperceptible, apical/PMI
RR	12–20 [17]	Tachypnea, bradypnea, apnea, dyspnea, SOB, shallow
BP	<120/80 [20]	Hypotension, orthostatic hypotension, elevated, Stage 1, Stage 2, crisis, systolic/diastolic, Korotkoff
SpO_2	**NOT IN SLIDES**	Pulse oximetry, SpO_2

PQRST pain assessment [Slide 24], for verbal, aware patients

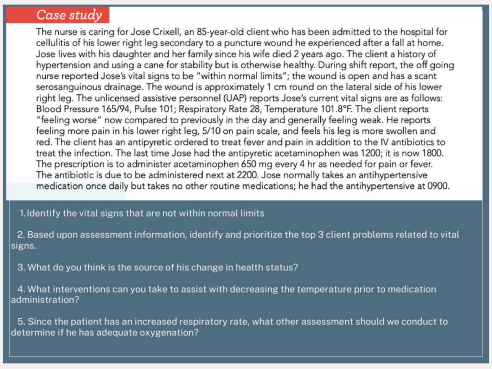
P	Provoking/relieving: what brings it on, what makes it better or worse, current meds and their effect, past treatments
Q	Quality: stabbing, burning, sharp, aching?
R	Region and radiation: where is it, and does it spread?
S	Severity: now, at worst, at least, most of the time; effect on daily activities
T	Time: onset, constant or intermittent, how often, how long

ABC prioritization

The FAQ lists ABC: **airway, breathing, circulation**. **NOT IN SLIDES** No slide teaches ABC. Her Slide 25 case (Jose, 85, cellulitis: BP 165/94, P 101, RR 28, T 101.8 °F, pain 5/10) asks you to prioritize the top 3 VS problems, which is where ABC thinking applies.



Slide 24: PQRST



Slide 25: case study

4 · Communication (Part 1) [Ch. 8 slides + Communication Scenario deck]

The communication process & influencing factors [Slides 3, 5–6, 13, 17]

Communication = exchanging information and generating and transmitting meaning between two or more people. It is linked to a healthy work environment (AACN) and to safe, quality care [3]. **Components [5]: sender (encoder)** starts it; **message** (visual, auditory, kinesthetic); **channel** (how it's sent); **receiver (decoder)**; **feedback** (the receiver's response back).

Levels [13]: intrapersonal (self-talk, which can be very damaging), interpersonal (two or more people), group (small group, organizational; group dynamics). **Influencing factors [17]:** developmental level, gender, sociocultural differences, **space and territory** (private zones: head, mouth, feet, face, neck, front of body, genitalia), roles and relationships, occupation/hobbies, physical/mental/emotional state, values, environment, **trust**.

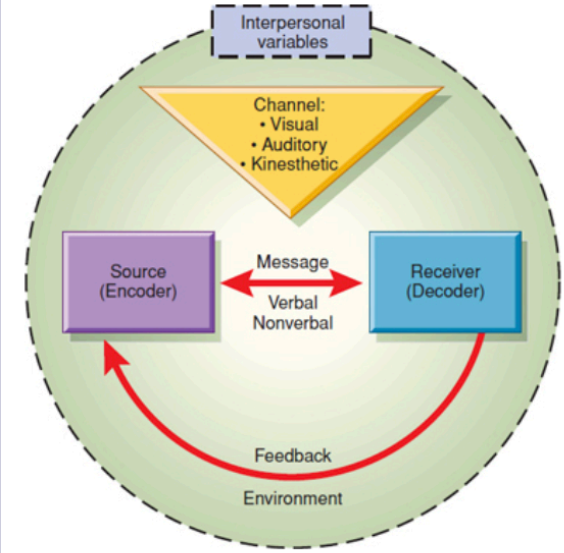


FIGURE 8-1. The various components in the process of communication.

Slide 6: communication process

Nonverbal communication [Slides 7, 11–12]

Verbal = spoken and relies on language. For non-English speakers, an interpreter is the priority. **Nonverbal**: touch, eye contact, facial expression, posture, gait, gestures, appearance/grooming, sounds, silence. **If verbal and nonverbal conflict, believe the nonverbal.** Other forms: electronic (social media, email, text) and paper. Her practice question [12]: stooped gait, grimacing and gasping point to **pain**.

Interpersonal competencies [Slides 21–22, 24]

Social relationship [21]	Helping (professional) relationship [22]
Mutual, spontaneous, both people's needs considered, no time limit, information shared equally	Meets the patient's needs, has a specific purpose and goals, three phases, a definite end, **unequal sharing of information**

Dispositional traits [24]: warmth/friendliness, openness, respect, empathy, honesty, authenticity, trustworthiness, caring, competence. **Empathy** = seeing things from the patient's point of view. **Sympathy** shifts the focus to the nurse, whose own feelings get projected onto the patient.

Phases of the therapeutic (helping) relationship [Slide 23]

Orientation	The patient calls the nurse by name and describes each person's role. Agree on goals, the location, frequency and length of contacts, and how long the relationship lasts.
Working	The patient actively takes part, cooperates toward mutually acceptable goals, and expresses feelings and concerns.
Termination	The patient helps identify goals met or progress made and talks about feelings about ending.

Effective vs ineffective communication [Slides 25, 32–33, 40]

Therapeutic ✓ [Slide 25]	Non-therapeutic ✗ [Slide 33]
Conversation skills (tone, clarity, truthfulness, open mind); active listening (sit, alert, relaxed, eye contact if appropriate); **silence**; touch (shoulder or hand, otherwise ask consent); humor used carefully. Questions: **open-ended** ("How are you feeling right now?"), **validating** ("You seem upset, can we talk about it?"), **clarifying** ("Did I understand you correctly…?"), **reflective** ("What went well today?"). Focus on feelings; **state observed behaviors** ("I can see you have been crying…"); reflect/restate/rephrase ("It sounds like you're frustrated…"); neutral responses; **exploring** ("Let's talk about it").	Closed-ended questions; forgetting they are human; failure to listen; **leading questions** ("You're not really sad, right?"); **giving advice**; **changing the subject**; **talking about yourself** ("My grandma also had cancer"); telling the patient they're wrong; **false reassurance**; **clichés** ("Everything happens for a reason"); **giving approval** ("I think you are making the right choice"); **asking why**; **defending other staff** ("Sally is actually a fantastic nurse…").

Her practice questions: the patient is silent after hearing a diagnosis, so **allow time to think** [32]. "I am sure everything will be fine" = **false reassurance** [40].

Role-play scenarios: technique vs barrier [Communication Scenario deck]

#	Patient	Therapeutic	Non-therapeutic
1	Angry, long ED wait	Reflective question/comment	Aggressive behavior
2	Nervous, IV start	Gentle, appropriate humor	Arguing
3	Hopelessness	Exploring statement (assess safety)	Failure to listen
4	Doesn't understand plan	Open-ended questions	Accusatory "why" questions
5	Anxious pre-op	Realistic reassurance and support	False reassurance/promises
6	Frustrated, chronic HTN	Active listening	Judgmental comments
7	Grieving family member	Emotional/spiritual support	Clichés
8	Confused elderly	Presenting reality, reorientation	Minimizing concerns
9	Isolated adolescent	Restating/mirroring	Nonverbal boredom
10	Refusing treatment	Clarifying statement	Disapproving comments
11	Overwhelmed new parent	Validating statement	Giving advice
12	Unexpected post-op pain	Direct, focused question	Leading question
13	Teen, body image	Empathetic statement	Intrusive/premature probing

14	Seriously ill, afraid of dying	Therapeutic touch, with permission	Talking about yourself
15	Behavior doesn't match pain	Making observations	Ignoring nonverbal cues
16	Transgender, respect	Affirming identity	Dehumanizing/dismissive
17	Parent of chronically ill child	Therapeutic silence	Only yes/no questions
18	Student near burnout	Collaborative problem-solving	Gossip/rumors
19	Parent learning diabetes care	Offering self and support	Poor professional presentation
20	Limited English	Qualified medical interpreter	Frustrated/dismissive

Interventions for impaired communication [Slides 41–43]

- **Visually impaired**: announce yourself and give your name; speak in a normal tone; explain before touching; say when you're leaving; call light within reach; orient them to room sounds and layout; glasses clean, contacts in.
- **Hearing impaired**: get their attention first (gentle touch or move into view); face them; simple sentences at a natural pace for lip reading; no gum or covering your mouth; pantomime, sign or write; hearing aids clean, working and in place.
- **Physical barrier (laryngectomy, ET tube)**: simple means such as eye blinks or hand squeezes for yes/no, writing pads, communication boards, flash cards, sign language; make sure everyone can use the method; be patient; make sure they can signal for help (call bell).
- **Cognitively impaired**: eye contact; quiet setting; simple, concrete steps without pronouns or abstract terms; **avoid open-ended questions and offer choices** ("brown pants or gray?"); if there's no response after **2 minutes**, repeat, then take a break.
- **Unconscious**: hearing is the **last sense lost**, so assume they can hear; talk normally; speak before touching; keep noise low.
- **Non-English speaking**: **use an interpreter whenever possible**; a translation dictionary; simple sentences in a normal tone; pantomime; watch nonverbal cues.

SBAR [Slide 44]: Situation, Background, Assessment, Recommendation. Used for hand-off and nurse-to-provider communication to make communication more effective.

Her Week 4 Wrap Up announcement says the Communication PPT is midterm material even though it wasn't covered in class.

Gaps: FAQ items her sources don't answer

- Hand hygiene times and when sanitizer isn't appropriate (Slide 24 asks without answering)
- PPE donning/doffing order
- HAI prevention specifics beyond hand hygiene, asepsis for Foleys and central lines, and device risk
- Step-by-step: ambulation with safe fall, wheelchair transfer, lateral transfer to gurney, repositioning in bed
- Temporal thermometer technique; older-adult and women temperature questions (Slide 6); which VS rises with fever (Slide 8)
- First-quarter scope for abnormal temperature (FAQ: recheck and notify)
- Tachycardia/bradycardia definitions and the "difficulty breathing lying down" term (blanks on the slides)
- Electronic VS machine operation; normal SpO_2 range
- High-Fowler's/orthopneic for SOB and O_2 out of scope (FAQ only)
- LLUSN manual 2-step BP checklist steps
- ABC prioritization